INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Lomustine + bevacizumab 7.5 mg/kg

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00804
Version 2

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1 and 22. Up to six cycles are planned, with review before each treatment.

Treatment stages

01 **Lomustine + bevacizumab 7.5 mg/kg**
42-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Lomustine + bevacizumab 7.5 mg/kg · National Cancer Control Programme (NCCP) 00804

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan. Call urgently for bleeding, severe abdominal pain, chest symptoms, new headache with visual changes or wound problems. Tell the team about planned surgery before treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

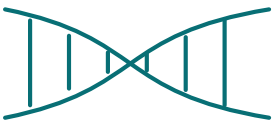
How your medicines work

Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Your team discusses likely benefits, risks, symptoms and quality-of-life goals.

ORAL CHEMOTHERAPY CAPSULE

Lomustine

CCNU



Damages deoxyribonucleic acid (the cell's genetic material) (DNA)

Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

What to watch for

This is one dose on Day 1, not a daily medicine. If an extra dose is taken, get urgent advice even if you feel well. Keep blood tests after the dose and report fever, bruising, bleeding, new cough or breathlessness.

TARGETED ANTIBODY GIVEN INTO A VEIN

Bevacizumab

Avastin / prescribed biosimilar



Blocks the blood-vessel growth signal VEGF

Bevacizumab binds VEGF, a signal involved in making and maintaining blood vessels. Blocking this signal can change the blood supply and leakiness of tumour vessels. It can also affect normal vessels, which helps explain high blood pressure, protein in the urine, bleeding, clots and slower wound healing. A scan change does not by itself prove that the tumour has stopped growing.

VEGF signal blocked → Tumour vessels affected → Response and safety reviewed

What to watch for

Report bleeding, severe abdominal pain, new chest or breathing symptoms, sudden neurological changes, headaches with visual changes, and any operation or wound problem. Keep blood-pressure and urine checks.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment and its purpose

Keep the exact diagnosis, regimen and treatment goal together.

What each medicine does

Lomustine: Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests. Bevacizumab: Bevacizumab binds VEGF, a signal involved in making and maintaining blood vessels. Blocking this signal can change the blood supply and leakiness of tumour vessels. It can also affect normal vessels, which helps explain high blood pressure, protein in the urine, bleeding, clots and slower wound healing. A scan change does not by itself prove that the tumour has stopped growing.

Your schedule

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1 and 22. Up to six cycles are planned, with review before each treatment.

Evidence and uncertainty

Adding bevacizumab delayed measured progression in the trial but did not improve overall survival. Treatment goals and burdens need an individual discussion. The trial used bevacizumab 10 mg/kg every two weeks, not this NCCP 7.5 mg/kg every three weeks schedule. No trial survival or response number is assigned to this dose schedule. Trial lomustine dose escalation and treatment duration do not replace the current NCCP regimen. Historical pathology, patient selection and later treatments limit individual prediction.

Consent and decisions

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. Keep steroid and seizure plans separate from anticancer treatment.

Understanding your particular glioma

Your diagnosis includes the tumour's cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

A glioma is a tumour arising in the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different types. Cancer that travels to the brain from another organ is a brain metastasis and follows a different pathway.

Grade is not the same as stage

Glioma reports combine the tumour type, grade and molecular findings. Grade is not a measure of spread around the body: a grade 4 brain tumour does not mean body-wide metastases. Ask your specialist to explain older terms in your records.

isocitrate dehydrogenase (IDH) and 1p/19q

Changes in the IDH genes and loss of chromosome regions 1p and 19q help classify certain gliomas. The pattern of results matters. These are usually tumour-sample tests; they do not by themselves show that a change runs in the family.

O6-methylguanine-DNA methyltransferase (MGMT) promoter methylation

The MGMT gene makes a protein that repairs a particular type of DNA damage. Methylation in its control region can reduce that repair and inform discussion of temozolomide benefit. This is a tumour-tissue result, not a blood marker or a personal survival percentage.

A useful diagnosis record

Keep the exact pathology diagnosis, sample date, grade and available molecular results together. Label a result as pending or not tested when appropriate. Ask whether further pathology review would affect the plan.

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Place biopsy or surgery, radiotherapy, medicines and follow-up on one plan. Concurrent means treatments overlap; adjuvant means treatment follows the main treatment. A new phase can require different doses, tests and support medicines.

Agree what treatment is trying to achieve

Ask what treatment can reasonably achieve for your tumour and which effects matter most to you. Discuss benefits, burdens, alternatives and the reasons for a pause or stop.

Put evidence in context

A study result applies to a particular group, diagnosis and schedule. Older studies may use different tumour names. Ask which evidence fits your plan; an average cannot forecast your response or lifespan.

Discuss options available to you

Ask about suitable trials, another specialist opinion and other treatment or symptom-care options. Your multidisciplinary team explains what is appropriate and available for you.

Make room for your priorities

Bring up thinking, independence, family time, work and the side effects you most want to avoid. Ask for time, written notes or a supporter if that helps you weigh the choices.

EORTC 26101: evidence and schedule limits

The randomised trial compared lomustine plus bevacizumab with lomustine alone after glioblastoma progression.

SACTCHECK · CHAPTER 02

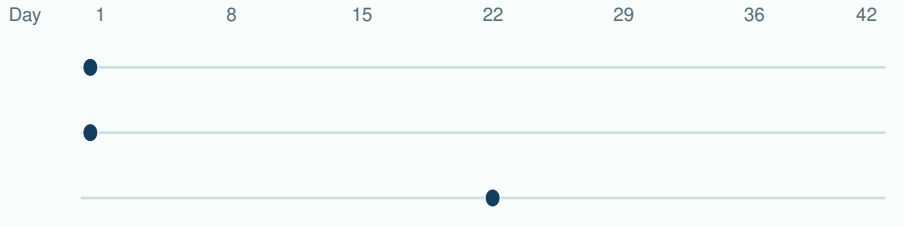
Your schedule, tests and appointments

Lomustine + bevacizumab 7.5 mg/kg · NCCP 00804

Your whole treatment course

Lomustine + bevacizumab 7.5 mg/kg

42-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

42-DAY CYCLE

Lomustine + bevacizumab 7.5 mg/kg

Up to 6 cycles | **42 days: lomustine once D1; bevacizumab intravenous (IV) D1 + 22.**

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1 and 22. Up to six cycles are planned, with review before each treatment.

Each start, visit, delay or dose change is confirmed by your team. The card records treatment and does not authorise another dose.

BASELINE → TREATMENT → FOLLOW-UP

Tests, symptoms and treatment decisions

Medicine-safety checks, tumour-tissue testing and response imaging answer different questions. Product-information additions are labelled. No universal blood tumour-marker panel or scan interval is implied.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00804 v2 · current regimen · Lomustine medac current product information · January 2026 · EMA Avastin product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Lomustine + bevacizumab 7.5 mg/kg: treatment and checks

NCCP 00804: recurrent malignant glioblastoma; bevacizumab use is off-label.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Next treatment / response	After a pause or stop
Bloods / medicine safety NCCP 00804	Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/ examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment.	Regular blood count, kidney/ liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment.	Keep additional tests even on days without an anticancer dose.	Team reviews current and previous results, symptoms and dose history.	Agree continuing checks; lomustine effects may persist.
Neurology / medicines / function NCCP 00804	Record usual symptoms, medicines, daily function and support needs.	Review headache, seizures, speech, strength, cognition and any steroid plan.	Report new changes early; do not wait for an magnetic resonance imaging (MRI).	Discuss treatment tolerance, priorities and the next authorised dose.	Agree rehabilitation, symptom support and contacts.
Response / appointments NCCP 00804	Confirm exact diagnosis and baseline imaging.	Imaging and clinical review follow the specialist plan.	Record bookings, result-review contacts and questions.	Interpret scans with symptoms, treatment effects and medicines.	Agree the next imaging and clinical review.

What each check is for

1 Lomustine + bevacizumab 7.5 mg/kg

42-day cycle

Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Your team discusses likely benefits, risks, symptoms and quality-of-life goals.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take lomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Bevacizumab is given into a vein. The usual first infusion lasts 90 minutes; later infusions may take 60 and then 30 minutes if tolerated. Checks and observation add time. Report flushing, rash, breathing difficulty, dizziness or other new symptoms immediately.

STEP 2

Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

STEP 3

Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

STEP 4

Surgery, procedures and wounds

Tell all treating teams that you receive bevacizumab. Major surgery needs an appropriate treatment interval and a fully healed wound before restarting; minor and dental procedures also need individual advice. Do not decide timing yourself or rely on the wallet stamp.

STEP 5

Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

STEP 6

Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

STEP 7

Appointments and transport

Before starting: Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. During treatment: Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. MRI and neurological review follow your individual treatment plan. Echocardiography is considered only when indicated, including previous anthracycline treatment or chest-wall radiotherapy. If taking warfarin, international normalised ratio (blood-clotting test) (INR) is checked weekly until stable and before subsequent cycles.

STEP 8

Help with the daily plan

Agree help with prescriptions, travel, calls and symptom notes if fatigue, memory, sight or weakness makes these difficult. Your preferences guide who is involved. Keep the printed plan and real urgent contacts together.

STEP 9

Off-label treatment and infusion decisions

Bevacizumab for glioblastoma is outside its licensed indication in Ireland. The team should explain the reason for using it, expected benefit, uncertainty and alternatives as part of consent. Any rapid infusion outside the standard schedule has a separate local consent and governance process.

Taking treatment and organising visits

Different medicines and support plans have different days.

Missed, vomited or held medicine

Medicine and supplement review

Off-label treatment and infusion decisions

Safety tests, support and urgent symptoms

A treatment-free day is still part of the care plan.

Lomustine nausea prevention

Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.

Bevacizumab support is separate

Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.

Blood pressure, urine and wound care

Keep prescribed blood-pressure medicine and urine checks. Follow the team’s plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.

Separate steroid, seizure and bowel plans

Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count measures red cells, white cells and platelets. Kidney and liver tests assess other aspects of treatment safety. Follow the blood-test schedule even when you feel well or the scan is reassuring.

MRI and neurological review

MRI gives detailed pictures of the brain. The team also checks symptoms, movement, speech, thinking and daily function. Before scanning, tell staff about implants, metal, kidney problems or previous contrast reactions, and ask for help if the scan worries you.

Changes on a scan need interpretation

Treatment-related changes can resemble tumour growth on MRI; this is called pseudoprogression. The specialist interprets the scan with symptoms and may need another scan or further assessment. Report worsening symptoms promptly rather than assuming a scan change is harmless.

An individual follow-up timetable

Record the next MRI and the contact who will explain it. Your diagnosis, treatment phase and previous results determine the interval. Other scans or blood markers are ordered for a specific reason, not automatically before every cycle.

Keep results with decisions

Mark tests as booked, done and reviewed so the next action is clear. Contact the named service about a missing result. Do not change chemotherapy or steroids in response to a report without instructions.

Symptoms and supportive care

Lomustine + bevacizumab 7.5 mg/kg · NCCP 00804

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Lomustine · one oral dose	 **Day 1** Take once on Day 1 only; use the newly authorised prescription.
Bevacizumab infusion	 **Day 1** Blood pressure, urine and safety review before treatment; observation during and after.
Bevacizumab infusion	 **Day 22** Blood pressure, urine and safety review before treatment; observation during and after.
Continuing blood-count checks	 **Days 1–42** Lomustine SmPC advises weekly counts for at least six weeks; arrange all checks.
Symptoms and support between visits	 **Days 1–42** Keep the written emergency plan and do not wait for the next treatment to report changes.

A treatment record is not a prescription: The team may delay, alter or stop a component after reviewing symptoms, blood tests, organ function and previous toxicity. Do not restart a held medicine or add a replacement dose yourself.

Lomustine is one dose, not daily: Only the authorised Day 1 dose belongs to this cycle. Do not take a further course before six weeks and a new team decision. Taking repeated daily doses can cause serious harm. If a dose is missed or vomited, contact the team or pharmacist before taking any more. If an extra dose is taken, seek urgent clinical advice straight away even if you feel well; do not wait for symptoms.

Delayed marrow effects: Current lomustine product information advises blood counts weekly for at least six weeks after a dose. Low counts may be delayed and cumulative, so keep checks even when you feel well or the next treatment is postponed.

Infusion visits and procedures: Bevacizumab needs review around surgery and wound healing. Tell the team before an operation, dental procedure or other intervention, and do not assume an infusion is safe because a calendar interval has passed.

Why these bands stay broad: Days without a scheduled dose still need symptom monitoring and planned tests. Bevacizumab infusion visits are not separate lomustine cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00804 v2 · current regimen ↗

There are no guaranteed "safe days". Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan.

Between-treatment notes

Day 1: Two medicines, different routes

Lomustine is a single oral dose; bevacizumab is an infusion. Keep the anti-sickness plan for lomustine. Confirm cycle number and the exact capsule supply before taking it.

Day 2–42: Separate infusion visits and continuing checks

Return for bevacizumab on Day 22. No extra lomustine is scheduled. Continue blood-pressure, urine and blood checks. Report symptoms and planned surgery; each medicine has its own safety decision.

Support around the medicines

1 

Blood pressure, urine and wound care

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Before treatment, note your usual walking, strength, speech, sight, memory, sleep and seizure pattern. Record new changes and their timing. A trusted person can help if you want them involved.

Steroids need a separate written plan

Steroids may reduce swelling around a brain tumour. If needed, ask for a written dose, review date and reduction plan; do not stop suddenly or adjust the dose yourself. Report infection symptoms, marked thirst, weakness or troublesome sleep or mood changes.

Seizure medicines and rescue instructions

Take prescribed seizure medicine on its own schedule. Ask what to do after a missed or vomited dose and how to manage a breakthrough seizure. Do not switch or stop it yourself. Anyone giving a rescue medicine needs the personal instructions and training.

Safety at home and outside

Discuss driving, work, bathing, swimming, heights and machinery with your team, taking account of symptoms and seizures. Individual medical and licensing advice is needed. Occupational therapy can help make daily routines safer.

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about changes in memory, language, attention, sight or planning. Rehabilitation can involve physiotherapy, occupational therapy, speech and language therapy or psychology. Ask for practical help with routines and medicines.

Food, nausea, bowels and energy

Use your prescribed nausea and bowel plan and report problems early. Ask for dietitian help if intake or weight falls. Adapt movement to your balance and strength, and ask the team to assess fatigue that disrupts daily life.

Emotional support and carers

Tell the team about changes in mood, behaviour or coping. Patients and carers may each need psychological or practical support. Agree who can receive information and ask for help before caring becomes overwhelming.

Work, transport and finances

Tell the nurse or social worker about travel, employment, finances and caring responsibilities. Arrange practical help with medicines or appointments if symptoms make those tasks difficult.

Supportive care and future wishes

Supportive and palliative care can help manage symptoms while treatment continues. When you are ready, discuss future care preferences and whom you want involved. Ask your team to help document these wishes.

Information that works on paper

Keep the latest paper schedule, medicine list, results plan and emergency contacts together. The treatment card helps others identify your regimen. It supports, but does not replace, the prescription and emergency instructions.

Questions worth asking

What is my exact diagnosis and how does this regimen fit?

NCCP 00804: recurrent malignant glioblastoma; bevacizumab use is off-label. Ask for the pathology, relevant molecular findings and the treatment goal in plain language.

How do I handle a missed, vomited or held dose?

Contact the oncology team or pharmacist for medicine-specific instructions. Do not double a dose or replace treatment that was held for toxicity.

Who helps with steroids, seizures, hearing, memory or daily tasks?

Ask for written medicine plans and a named contact. Rehabilitation, practical support and a trusted helper may support the daily plan.

When do we review whether treatment is helping?

Agree the next clinical and imaging review, what would count as benefit, how side effects will be weighed and what would lead to continuing, changing or stopping.

> **Try explaining the plan back:** Can you show the day for each medicine, your next safety tests, the current support plan and whom to call for fever, new neurological symptoms or another urgent change?

Lomustine + bev 7.5 NCCP 00804: recurrent malignant glioblastoma; bevacizumab use is off-label. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · lomustine and bevacizumab

Active medicines: Lomustine + Bevacizumab

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	At least 30 minutes before the single Day1 lomustine dose; record delayed-emesis cover and rescue duration NCCP V6 Table5 p8 and timing p4. Review QT risk, liver function, constipation and interactions. Lomustine is a single dose, not a daily course.

Additional phase-specific support

- Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.
- Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.
- Keep prescribed blood-pressure medicine and urine checks. Follow the team’s plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.
- Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 22 · bevacizumab only

Active medicines: Bevacizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.
- Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.
- Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.
- This is a bevacizumab-only visit. Do not repeat the Day 1 lomustine dose or automatically repeat its antiemetic course.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Brain / blood pressure

Bevacizumab / tumour

Keep BP checks. Severe headache, visual change, seizure, sudden weakness or speech difficulty needs urgent assessment.

3 Chest / breathing

Bevacizumab / lomustine

New breathlessness, cough or chest pain needs prompt advice. Severe breathing difficulty needs emergency help.

5 Kidneys / urine

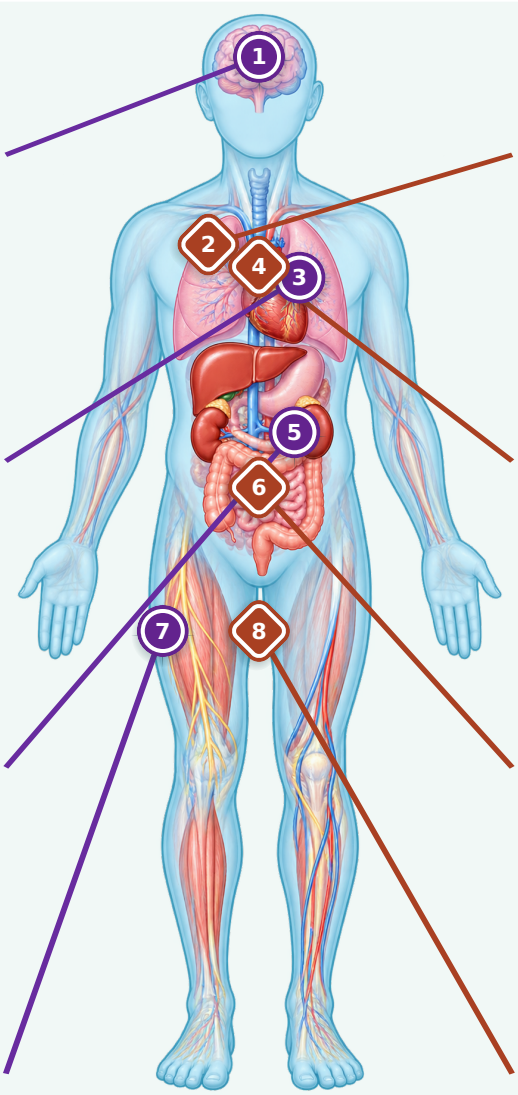
Bevacizumab

Protein in urine may cause no early symptoms. Keep urine and kidney checks; report swelling, reduced urine or a major change.

7 Skin / wound healing

Bevacizumab

Tell the team about operations, dental procedures and wounds. Report wounds that open, fail to heal or show infection.



2 Blood / bleeding

Bevacizumab / lomustine

Report unusual bruising or bleeding. Heavy bleeding, sudden chest pain or collapse needs emergency help. Counts can fall weeks after lomustine.

4 Heart / vessels

Bevacizumab

New chest pain, a painful swollen leg or sudden one-sided weakness needs urgent advice. Severe chest pain or collapse needs emergency help.

6 Abdomen / bowel

Bevacizumab / lomustine

Severe or persistent abdominal pain, vomiting or blood in stool needs urgent advice. Follow the prescribed nausea and bowel plan.

8 Fertility / pregnancy

Anticancer treatment

Discuss fertility and contraception before treatment. Do not breastfeed during therapy; obtain written medicine-specific advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call urgently for bleeding, severe abdominal pain, chest symptoms, new headache with visual changes or wound problems. Tell the team about planned surgery before treatment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Sources and further information

Lomustine + bevacizumab 7.5 mg/kg · National Cancer Control Programme (NCCP) 00804

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Complete the individual local plan and interaction review before use; clinical/pharmacy approval pending**
   https://healthservice.hse.ie/documents/6872/804_V2_Lomustine_and_bevacizumab_7.5.pdf
2. **Lomustine medac current product information · January 2026**
   https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf
3. **EMA Avastin product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf
4. **NCI: adult central nervous system tumours · diagnosis and grade education**
   https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq
5. **NHS: dexamethasone · use and stopping safely**
   https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/
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